

Policy on Patient and Family Rights in Care and Decision-Making

«Hospital Name»

Document control

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Prepared by	Patient Rights Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» respects patient and family values, beliefs, dignity, privacy and confidentiality; protects patients from neglect or abuse; obtains informed consent before high-risk care; and supports the patient's and family's right to refuse treatment, seek a second opinion, complain, know treatment costs, access records, and be informed of their care.

1. Purpose

This policy explains how «Hospital Name» supports patient and family rights that involve individual beliefs, values, dignity, privacy, confidentiality, protection from neglect or abuse, informed consent before high-risk care, refusal of treatment, second opinion, complaints, cost information, record access, and information-sharing preferences.

The detailed informed-consent process, complaint redressal mechanism, and cost-estimate procedure are covered in the hospital's other policies.

2. Scope

This policy applies to all clinical and nursing staff involved in examination, procedures, and treatment, and to all patients and families receiving care at «Hospital Name».

3. Policy standards

«Hospital Name» respects patient and family values, beliefs, cultural needs and spiritual requests; protects dignity and privacy during examination and procedures; protects patients from neglect and abuse, especially vulnerable patients; and treats patient information as confidential.

Informed consent is obtained before transfusion, anaesthesia, surgery, research, or other invasive/high-risk procedures; patients may refuse treatment, seek a second opinion, complain, and know treatment costs; and patients may access their clinical records and determine what information is shared with family.

4. Non-negotiable rules

- Do not continue an examination, procedure or treatment that exposes the patient beyond what the procedure requires, or that leaves the patient without privacy.
- Do not leave a patient in a situation of neglect or abuse — stop the unsafe situation, protect the patient, and report it the same shift.
- Do not start transfusion of blood or blood components, anaesthesia, surgery, a research protocol, or any other invasive/high-risk procedure or treatment without informed consent, except where the documented emergency life-saving rule applies.
- Do not reveal confidential patient information, including HIV status, without the patient's permission, and do not write or display confidential information where the public can read it.
- Do not photograph or record a procedure without explicit informed consent, and do not reveal the patient's identity in any such material.
- Do not block a patient or family from seeking a second opinion from within or outside the organisation.

5. What we do

5.1 Respect values, beliefs, cultural and spiritual needs

Patient and family rights include respecting values and beliefs, special preferences, cultural needs, and responding to requests for spiritual needs — including, where relevant, how the patient wishes to be addressed, dietary preferences, worship requirements, and any specific requirement following death.

5.2 Protect dignity and privacy during examination and procedures

Staff ensure that the patient's privacy and dignity are maintained during all stages of care. «Hospital Name» has written guidelines for maintaining privacy and dignity. The patient is exposed only just before the actual procedure. Explicit informed consent is taken for photographs or recordings of procedures, and the patient's identity is not revealed.

5.3 Protect patients from neglect and abuse

Patient and family rights include protection from neglect or abuse. Special precautions are taken for vulnerable patients — including elderly, neonates, physically and mentally challenged patients, comatose patients, and patients under anaesthesia.

5.4 Treat patient information as confidential

Effective measures are taken to maintain the confidentiality of all patient-related information. Staff avoid patient-related discussions in public places. Statutory requirements regarding privileged communication are followed. Confidential information, including HIV status, is not revealed without the patient's permission, and is not written or displayed where the public can see it.

5.5 Respect the right to refuse treatment

The treating doctor discusses all available options and allows the patient to make an informed choice. In case of refusal, the treating doctor explains the consequences of refusal and documents the discussion. After explanation, if the patient still refuses treatment, the refusal is respected.

5.6 Support the right to a second opinion

A mechanism exists for patients and families to seek a second opinion from within or outside the organisation. «Hospital Name» respects the patient's and family's decision and facilitates access to relevant information or clinical evaluation when a second opinion is sought.

5.7 Obtain informed consent before high-risk care

Informed consent is obtained before transfusion of blood and blood components, anaesthesia, surgery, initiation of any research protocol, and any other invasive/high-risk procedure or treatment. Consent is obtained by the treating doctor or a doctor member of the treating team. Those requirements are covered in the hospital's other policies.

5.8 Support the right to complain

Displayed patient rights include the right to make a complaint and the methodology to voice it. The complaint mechanism is accessible, and redressal of complaints is fair and transparent. Those requirements are covered in the hospital's other policies.

5.9 Provide cost information

Patients and families are explained about the expected costs of treatment — including consultations, procedures and investigations — in a transparent manner. Those requirements are covered in the hospital's other policies.

5.10 Provide access to clinical records

Every patient has access to their clinical record, in consonance with the Code of Medical Ethics and applicable statutory requirements. Those requirements are covered in the hospital's other policies.

5.11 Discuss the treating doctor, care plan and progress

Patients and families are given information on the name of the treating doctor, care plan, progress, and healthcare needs, discussed with them directly.

5.12 Support patient control over information sharing

A mechanism exists for the patient to determine what information about their care is provided to self and to family. For minors, information is provided to at least one parent or guardian.

6. Stop-work authority

Do not continue an examination, procedure or treatment that exposes the patient beyond what the procedure requires, or that leaves the patient without privacy (screens, drapes, closed door or equivalent).

Do not leave a patient in a situation of neglect or abuse (unattended fall risk, unwarranted repeated examination, manhandling, or failure to protect a vulnerable patient). Stop the unsafe situation, protect the patient, and report it the same shift.

Do not start transfusion of blood or blood components, anaesthesia, surgery, a research protocol, or any other invasive/high-risk procedure/treatment without informed consent obtained under the hospital's informed consent policy, except where the documented emergency life-saving rule applies.

Stop-work applies to the examination or procedure start. Immediate life-saving measures continue while escalation happens, and are documented.

The person who stops tells the treating doctor and the Medical Superintendent the same shift. Refusing an unsafe examination or a procedure without consent is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring patient rights in care and decision-making are respected across the organisation; receives stop-work escalations.
Patient Rights Officer	Owns implementation of this policy; maintains privacy/confidentiality guidelines, awareness records, and second-opinion and complaint mechanisms.
Treating Doctor	Obtains informed consent for high-risk care; discusses refusal of treatment and its consequences; documents all such discussions.

Nursing Superintendent	Ensures staff maintain patient privacy, dignity, and confidentiality in daily practice; escalates stop-work triggers.
Quality Coordinator	Audits this policy; holds training records, staff acknowledgements, and stop-work event logs.

8. Quality monitoring

Monitoring area	What is monitored
Privacy and dignity	Privacy guidelines on file; audit/observation records of privacy practice; consent records for photography.
Neglect/abuse protection	Incident reports; vulnerable-patient protection records; stop-work event logs.
Confidentiality	Confidentiality guidelines; staff training records; incident records involving disclosure.
Informed consent for high-risk care	Consent forms for each listed act; stop-work event log if triggered.
Refusal of treatment	Medical record entries documenting refusal discussions and outcomes.
Second opinion and complaints	Records of facilitated second-opinion access; complaint mechanism records.
Records access and information sharing	Access records; documented patient preferences on information sharing.

9. Training and staff acknowledgement

All clinical and nursing staff shall be familiar with patient privacy, dignity, confidentiality, neglect/abuse protection, informed consent for high-risk care, and the stop-work triggers in this policy, at induction and periodically.

I have read the Policy on Patient and Family Rights in Care and Decision-Making of «Hospital Name» and understand my responsibilities under it.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all clinical and nursing staff, the Patient Rights Officer, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
HIV	Human Immunodeficiency Virus
NABH	National Accreditation Board for Hospitals and Healthcare Providers

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
PRE.2.a	Commitment	Sections 3 and 5.1 address respect for values, beliefs, cultural needs and spiritual requests.
PRE.2.b	Commitment	Sections 3, 5.2 and 6 address privacy, dignity, and the stop-work trigger for exposure/privacy breach.
PRE.2.c	Commitment	Sections 3, 5.3 and 6 address protection from neglect/abuse and the stop-work trigger for active neglect/abuse.
PRE.2.d	CORE	Sections 3 and 5.4 address confidentiality of patient information, including HIV status.
PRE.2.e	Commitment	Section 5.5 addresses the right to refuse treatment.
PRE.2.f	Commitment	Section 5.6 addresses the right to a second opinion.
PRE.2.g	CORE	Sections 3, 5.7 and 6 address informed consent before high-risk care and the related stop-work trigger.
PRE.2.h	Commitment	Section 5.8 addresses the right to complain.
PRE.2.i	Commitment	Section 5.9 addresses cost information.
PRE.2.j	Commitment	Section 5.10 addresses access to clinical records.
PRE.2.k	Commitment	Section 5.11 addresses information on treating

		doctor, care plan and progress.
PRE.2.l	Achievement	Section 5.12 addresses patient control over information sharing with family.

13. Required Records/Evidence Checklist

Values, beliefs and cultural needs — PRE.2.a (Commitment)

- Awareness records and preferences noted in the admission sheet/nursing kardex.

Privacy and dignity — PRE.2.b (Commitment)

- Privacy guidelines document.
- Consent forms for photography/recording of procedures.
- Audit/observation records of privacy practice.
- Stop-work event log for exposure/privacy triggers.

Neglect and abuse protection — PRE.2.c (Commitment)

- Incident reports for neglect/abuse events.
- Vulnerable-patient protection records.
- Stop-work event log for active neglect/abuse triggers.

Confidentiality — PRE.2.d (CORE)

- Confidentiality guidelines.
- Staff training records on confidentiality.
- Incident records involving disclosure, if any.

Refusal of treatment — PRE.2.e (Commitment)

- Medical record entries documenting options offered, consequences explained, and sustained refusal.

Second opinion — PRE.2.f (Commitment)

- Written mechanism for seeking a second opinion.
- Records of facilitated second-opinion access.

Informed consent for high-risk care — PRE.2.g (CORE)

- Consent forms on file for transfusion, anaesthesia, surgery, research, and other high-risk acts.
- Stop-work event log for missing-consent triggers.

Complaints — PRE.2.h (Commitment)

- Display boards including complaint information.

- Complaint mechanism description.

Cost information — PRE.2.i (Commitment)

- Records of cost explanation or written estimates provided to patients.
- Tariff availability mechanism.

Records access — PRE.2.j (Commitment)

- Written record-access procedure.
- Records of access provided to patients.

Treating doctor and care-plan information — PRE.2.k (Commitment)

- Admission notes and clinical records naming the treating doctor and documenting care-plan discussion.

Information-sharing preferences — PRE.2.l (Achievement)

- Written mechanism for patients to determine information sharing with family.
- Records of patient-expressed preferences, including for minors.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PRE.2.
2. Code of Medical Ethics, Medical Council of India.

Disclaimer

This policy reorganises the supplied PRE.2 objective-element wording and Guidebook interpretation into plain-language guidance for «Hospital Name». It does not replace the NABH Accreditation Standards for Hospitals, 6th Edition, or the Guidebook, which remain the authoritative source in case of any conflict.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name» remains separately responsible for meeting all applicable statutory and regulatory requirements.

This policy shall be reviewed at least once every year, or earlier if NABH standards, the Code of Medical Ethics, or applicable law changes.

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